

Will AI Replace Doctors? The 2026 Medicine Risk Score

The free sampler — the score by track, the six factors in brief, and the honest reasons to think twice. The complete profile goes deeper.

2.1

AI EXPOSURE · SURGERY

where 10 is most at risk

Re-scored every six months. We publish where we might be wrong.

Quick answer: Medicine is among the best-protected careers we track, but the score depends heavily on specialty. Surgery scores **2.1 out of 10** for AI exposure — the lowest of any career in this index. Primary care scores **2.9**. Radiology scores **4.9**, and it is the fastest-moving specialty we measure. **The protection in medicine is physical presence and human accountability, and specialties that rely on neither are moving.**

The short answer for parents: yes, medicine remains one of the safest paths available — but for the first time, choice of specialty carries an AI dimension. A student entering medical school today will choose a specialty in five or six years, and the specialties are no longer moving together.

Medicine AI risk score by specialty

Every career in this index is scored 1–10, where **10 is most exposed** to AI. Same six factors, same weights, applied identically to a surgeon and a paralegal.

SPECIALTY	2023	2025	FALL 2026	3-YR MOVE	BAND
Surgery	1.8	1.9	2.1	+0.3	Very low
Emergency medicine	2.0	2.2	2.5	+0.5	Very low
Primary care / family medicine	2.4	2.6	2.9	+0.5	Low
Anesthesiology	2.7	2.9	3.1	+0.4	Low
Psychiatry	3.1	3.4	3.7	+0.6	Low
Pathology	3.8	4.3	4.7	+0.9	Moderate
Radiology	3.9	4.4	4.9	+1.0	Moderate

2023 and 2025 figures are reconstructed using current methodology, not archived from past editions.

How that compares: the median career in this edition scores around 5.5. Bedside nursing scores 2.8. Physical therapy scores 2.5. Entry-level software development scores 8.1.

Surgery at 2.1 is the lowest score in the entire index — lower than nursing, the trades, or any other career we measure.

The spread is the story. Three years ago the gap between the safest and most exposed medical specialty was 2.1 points. It is now 2.8, and every year it widens. Medicine is not becoming riskier. It is becoming less uniform.

Will AI replace doctors?

No. But it is already changing what several specialties do all day.

The pattern is consistent across the profession: AI is absorbing the parts of medicine that involve reading, matching and documenting, and leaving the parts that involve a body in a room and a human being accountable.

AI IS TAKING	IT CAN'T TOUCH
Clinical documentation and letters	Physical examination
Coding and billing	Procedures and surgery
Guideline and drug interaction lookup	Deciding under genuine uncertainty
First-pass image reading	Owning the outcome when it goes wrong
Record summarization	Telling a family bad news
Triage and risk stratification	Judgment when the patient doesn't match the textbook

For most doctors this is welcome. Documentation burden is among the most-cited causes of physician burnout, and ambient systems that draft the note from the consultation are already in routine use.

The specialties where it cuts deeper are the ones whose core work is reading and matching.

Why does medicine score so low? The six factors

How primary care rates against each. Ratings are 0–10 on each factor's own terms.

FACTOR	RATING	EFFECT ON RISK
How much of this job can AI already do?	6.0	↑ moderate exposure
How hard will it be to land that first job?	1.5	↓ strongest protection in the index
Does it have to be done in person, with your hands?	8.0	↓ strong protection
Does someone need a human they can trust and hold responsible?	9.5	↓ strong protection
Does the law require a licensed human?	9.5	↓ strong protection
How often does the job hit genuinely new, high-stakes situations?	8.5	↓ strong protection

Medicine holds the strongest protection stack of any career we score. What varies between specialties is the first factor.

ONE FACTOR, WORKED THROUGH IN FULL

So you can see what the analysis actually looks like.

How much of this job can AI already do? — the factor that separates the specialties

This factor carries 35% of the score, more than any other, and it is where medicine's internal spread comes from.

Primary care rates 6.0. A substantial share of the work is documentation, coding, correspondence and guideline application — all of it automating quickly. What resists is the physical examination, the relationship built over years, and the judgment call on a patient who presents atypically. The automatable share is large but it is the administrative shell rather than the clinical core.

Surgery rates 4.5. Planning, imaging review and documentation are automatable. The operation is not, and nothing on the visible horizon changes that. This is why surgery sits at the bottom of the entire index.

Radiology rates 8.3, up from 5.6 in 2023. Image interpretation is pattern recognition on structured visual data, which is the single thing AI does best. Around three-quarters of all FDA-authorized AI medical devices are in radiology — the technology is concentrated there because that is where it works. The radiologist's job is shifting from reading to supervising, adjudicating complex cases, and owning the decision.

That is a 3.8-point spread on one factor, inside one profession, held by people with the same medical degree. It is the clearest example in this index of why scoring professions as single units hides the thing that matters.

Which medical specialties are safest from AI?

Ranked by exposure, safest first:

1. **Surgery** — 2.1. Physical, procedural, high-stakes, irreducibly hands-on. 2. **Emergency medicine** — 2.5. Continuous novelty, undifferentiated presentations, physical assessment under time pressure. 3. **Primary care** — 2.9. Long-term relationships, physical examination, and the diagnostic problem of the patient who doesn't fit. 4. **Anesthesiology** — 3.1. Physical and high-stakes, but more monitoring-based and protocol-driven than the specialties above. 5. **Psychiatry** — 3.7. Almost no physical component, but the deepest trust requirement in medicine and heavy regulatory protection. 6. **Pathology** — 4.7. Slide and sample interpretation is pattern work, though with more procedural and consultative content than radiology. 7. **Radiology** — 4.9. The most exposed specialty, and the fastest-moving.

The pattern holds across every profession we score: **move the same work behind a screen and exposure roughly doubles**. Nursing goes from 2.8 at a bedside to 5.4 at a desk. Teaching goes from 3.6 in a classroom to 6.8 online.

Is radiology a dead end?

This is the question we get asked most, and our honest answer is that we are less confident about this score than any other in the index.

The case for concern is real. Image interpretation is exactly what AI does well, adoption is furthest advanced here, and the score has moved a full point in three years — faster than any other protected career we track.

But the labor market has done the opposite of what the narrative predicts. Radiology posts sit unfilled, residency programs match at close to 100%, imaging volumes keep rising, and the shortage has deepened over the same decade in which displacement was being confidently forecast.

Both things are true, and the gap between them is wider in radiology than anywhere else we score. We publish the number with the disagreement stated rather than adjusting it to match the employment data — because bending a score to fit an outcome would make every trend in this index meaningless.

The Career Value Guide sets out both cases properly, including the strongest published argument against our position.

Is medicine worth it in 2026?

The AI question and the career question are different, and this index only answers the first.

Medicine scores exceptionally well on exposure, and its entry route — residency — is the most protected on-ramp we measure, mandated and structurally impossible to automate away.

The reasons people hesitate about medicine have not changed and have nothing to do with AI: it is a decade or more to independent practice, the training is expensive, early-career hours are punishing, and most systems require relocating repeatedly for training posts. Burnout affects around 42% of physicians — high, though it has fallen three years running, from 48.2% in 2023. A fifteen-year-old should understand that timeline concretely before committing to it.

A low AI score does not mean an easy path. The Career Value Guide covers the downsides honestly, alongside what doctors themselves say about the work.

Common questions

Will AI replace radiologists? Not on current evidence, though it is reshaping the job substantially. Radiology scores 4.9 — the most exposed medical specialty — while simultaneously running a persistent staffing shortage. The role is shifting toward supervision and adjudication rather than disappearing.

Which medical specialty is safest from AI? Surgery, at 2.1, which is also the lowest score of any career in this index. Emergency medicine and primary care follow closely.

Is medicine safer than nursing from AI? They are close. Primary care scores 2.9 and bedside nursing 2.8 — effectively identical on exposure. The differences that matter between them are training length, cost, and the nature of the work rather than AI risk.

Should my child avoid radiology because of AI? We would not say avoid. We would say choose it knowing the job in 2040 will look different from the job today, and that the evidence currently points in two directions at once.

Is medical school still worth it with AI? On exposure alone, yes — medicine holds the strongest protection stack we score. The harder questions are the ones that always applied: the length of training, the cost, and whether the work suits the person.

Related careers

- **Nursing** — 2.8 bedside, 5.4 desk-based
 - **Physical therapy and allied health** — 2.5, among the lowest in this edition
 - **Pharmacy** — 4.9, the surprise of the healthcare group
 - **Psychology** — 3.5 licensed clinical, 7.1 with the degree alone
 - **Computer science** — 8.1 entry-level, 5.4 senior. Free to read in full.
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What's in the medicine Career Value Guide

This sampler tells you where medicine stands. The full Career Value Guide tells you what to do about it.

	SAMPLER	CAREER VALUE GUIDE
Verdict, all specialty scores, 3-year trend	✓	✓
Six-factor ratings	✓	✓
Reasoning behind every factor rating	one example	✓
How durable each protection is — where AI is already pressing		✓
The radiology debate in full — including the strongest case against our score		✓
The honest downsides — training length, cost, attrition, burnout		✓
What's genuinely good about it — what doctors say, and the satisfaction data		✓
Who this work suits, and who it doesn't		✓
The AI-native advantage — how to prepare, and why this cohort has an opening		✓
Routes in — direct entry, graduate entry, international routes		✓
Choosing a specialty — how the AI dimension should and shouldn't weigh		✓
Program evaluation checklist		✓
Questions to ask a medical school — plus red flags		✓
Sourced further reading		✓
Discussion questions for parent and student		✓
A short version written directly to the student		✓
Technical scoring appendix		✓

GET THE CAREER VALUE GUIDE

Most families are weighing two or three careers seriously, and a few more they haven't ruled out. Pick the ones you need.

1 career	\$49	Choose →
3 careers	\$69	Choose →
Unlimited	\$99	Choose →

Each Career Value Guide includes everything in the table above, plus a short version written directly to the student and the technical scoring appendix. This edition and the next are included — we re-score every six months, so what you buy stays current for a year.

Still exploring? The *Atlas* covers whole fields rather than single careers — seventeen jobs that help people, mapped and compared, written for 13–16 year olds who haven't narrowed yet. Same pricing: 1 guide \$19, 3 guides \$29, all seven \$39.

[Read a complete Career Value Guide free →](#) We publish one Career Value Guide in full, openly, so you can judge the depth before buying anything. Computer science is the one to read — it's the most surprising result in the index.

Pivotum scores thirty careers against AI exposure using a fixed, published methodology, re-scored every six months. Scores measure exposure to what AI can already do — not how much any particular employer has deployed. We publish where we might be wrong.